Ayushman Bharat PM-JAY

Comprehensive Question Bank

100+ Questions | All 3 Stages | Levels 1–4 | Q&A Format

For Students, Researchers & Ayushman Mitras

STAGE 1 – Eligibility & Identification

Level 1 – Simple (Basic Concepts)

Q1. What is PM-JAY?

➤ Pradhan Mantri Jan Arogya Yojana (PM-JAY) is India's largest government-funded health insurance scheme offering up to ₹5 lakh per family per year for hospitalisation at empanelled hospitals — free of cost for eligible beneficiaries.

Q2. When and where was PM-JAY launched?

➤ PM-JAY was launched on 23rd September 2018 in Ranchi, Jharkhand, by Prime Minister Narendra Modi.

Q3. What does SECC 2011 stand for?

➤ Socio-Economic Caste Census 2011 — the primary database used by the government to identify PM-JAY beneficiaries.

Q4. What is the maximum coverage per family per year?

➤ Up to ₹5 lakh per family per year for secondary and tertiary hospitalisation.

Q5. Is there a restriction on family size?

➤ No. PM-JAY covers the entire family with no cap on family size or number of members.

Q6. Who manages PM-JAY at the national level?

➤ The National Health Authority (NHA) manages PM-JAY at the national level.

Q7. What is a State Health Agency (SHA)?

➤ SHA is the state-level body responsible for implementing PM-JAY within each state, managing empanelment, claims, and outreach.

Q8. What type of hospitalisation does PM-JAY cover?

➤ PM-JAY covers secondary and tertiary hospitalisation — meaning in-patient treatment requiring hospital admission for at least one night.

Q9. How many families does PM-JAY aim to cover?

➤ PM-JAY aims to cover approximately 10.74 crore (107.4 million) poor and vulnerable families across India.

Q10. Can PM-JAY be used in any hospital across India?

➤ Only in empanelled hospitals — government and private hospitals that have been approved under the PM-JAY scheme.

Q11. What does 'pre-existing conditions' mean in PM-JAY?

➤ PM-JAY covers all pre-existing diseases from day one — there is no waiting period unlike most private insurance schemes.

Q12. Is PM-JAY only for people below the poverty line?

➤ Primarily yes, but eligibility is based on SECC 2011 deprivation criteria, not just income. Some occupation-based urban poor categories are also included.

Q13. Who bears the cost of treatment under PM-JAY?

➤ The central and state governments jointly bear the cost. The ratio is generally 60:40 (Centre:State).

Q14. What is the helpline number for PM-JAY?

➤ 14555 — a toll-free number available in multiple languages.

Q15. Does PM-JAY cover outpatient (OPD) treatment?

➤ No. PM-JAY currently covers only in-patient (IPD) hospitalisation, not OPD or day-care visits below one night.

★★ Level 2 – Medium (Concept + Process)

Q16. What are the rural deprivation criteria under SECC 2011 for PM-JAY eligibility?

➤ Rural eligibility includes: households without shelter, destitute/living on alms, manual scavengers, primitive tribal groups, legally released bonded labourers, and households with no adult member between 16–59 years.

Q17. What are the urban occupational categories eligible for PM-JAY?

➤ Urban eligibility includes: rag pickers, beggars, domestic workers, street vendors, construction workers, sanitation workers, home-based workers, artisans, transport workers, and casual labourers.

Q18. How does the government identify eligible families without them applying?

➤ Using SECC 2011 data cross-referenced with ration card and other welfare databases, families are pre-identified and loaded into the PM-JAY beneficiary system automatically.

Q19. What is the difference between AB-PMJAY and state government health schemes?

➤ AB-PMJAY is a central scheme funded jointly. Many states have their own schemes (e.g., Tamil Nadu's CM Comprehensive Health Insurance) that run alongside or integrate with PM-JAY to expand coverage.

Q20. Can a family with a BPL ration card automatically get PM-JAY?

➤ Not automatically. Eligibility is based on SECC 2011 data, not just BPL card. However, many BPL families are covered since they often meet SECC deprivation criteria.

Q21. What documents does a person need to verify eligibility?

➤ Aadhaar card, ration card, mobile number, and sometimes caste or income certificate as required by the state.

Q22. What happens if a person's name is in the SECC data but has a spelling error?

➤ They can visit the nearest CSC or district health office to file a grievance and request data correction. Aadhaar-based verification helps resolve minor spelling mismatches.

Q23. Is PM-JAY portable across states?

➤ Yes. PM-JAY is fully portable. Beneficiaries can avail cashless treatment at any empanelled hospital across India, regardless of which state they registered in.

Q24. What are the key differences between PM-JAY rural and urban eligibility?

➤ Rural: based on deprivation criteria (housing, livelihood, social exclusion). Urban: based on occupational categories. Both use SECC 2011 as the base database.

Q25. How can a person know which hospitals near them are empanelled?

➤ Through the PM-JAY website (pmjay.gov.in), the Ayushman Bharat app, or by calling 14555 to find empanelled hospitals nearby.

Q26. Can migrants avail PM-JAY benefits outside their home state?

➤ Yes. Since PM-JAY is portable, migrant workers can access benefits at any empanelled hospital in any state — a key advantage for India's mobile workforce.

Q27. What is the role of UIDAI in PM-JAY eligibility verification?

➤ UIDAI (Unique Identification Authority of India) manages Aadhaar infrastructure. PM-JAY uses Aadhaar for identity verification during eligibility checks and eKYC.

Q28. Which states have NOT adopted PM-JAY?

➤ A few states like Odisha (initially) and West Bengal opted out or delayed adoption due to political reasons, running their own state schemes instead.

Q29. How does a pregnant woman or newborn get included in PM-JAY coverage?

➤ The scheme covers the family unit. Newborns are automatically eligible under the family's coverage. Maternity benefits are included under defined health packages.

Q30. What is the difference between secondary and tertiary care under PM-JAY?

➤ Secondary care: treatments at district hospitals for common conditions like fractures, infections. Tertiary care: complex treatments at specialised hospitals like cancer surgery, cardiac procedures, organ transplants.

★★★ Level 3 – Scenario Based

Q31. Ravi is a construction worker in Chennai earning ₹9,000/month with no BPL card. Is he eligible?

➤ Possibly yes. Construction workers are an urban occupational category under PM-JAY. He should check his name on the app using his Aadhaar or ration card. His state may have additional provisions.

Q32. A family moved from rural Bihar to Mumbai 4 years ago. Their SECC data shows their Bihar village address. Can they use PM-JAY in Mumbai?

➤ Yes. PM-JAY is portable across India. As long as their family is in the beneficiary database, they can use the card at any empanelled hospital in Mumbai without updating their address.

Q33. Meena's name is in the system but her husband's and children's names are missing. What should she do?

➤ She should visit the nearest CSC or district health office, carry all Aadhaar cards and ration card, and request family member addition. If denied, she can escalate via helpline 14555.

Q34. A man checks the PM-JAY app and finds three different families with his same name in his district. How does he confirm which entry is his?

➤ He can cross-check using additional identifiers like father's name, date of birth, and ration card number. Aadhaar-based eKYC will further confirm the correct match to his biometric identity.

Q35. A tribal family in Jharkhand has no ration card or Aadhaar. Are they excluded from PM-JAY?

➤ They may still be in the SECC 2011 database if surveyed. However, without Aadhaar they cannot complete eKYC. The priority action is to enrol in Aadhaar first through a camp, then check PM-JAY eligibility.

Q36. Sunita's family is eligible but their annual benefit of ₹5 lakh was exhausted due to a major surgery. Can they still get treatment?

➤ The ₹5 lakh limit resets every year. If the limit for the current year is exhausted, they must wait for the next year's renewal or explore state government supplementary schemes for additional coverage.

Q37. A daily wage labourer says 'I don't own anything, I should be eligible' but his name isn't found in the system. Why might this happen?

➤ PM-JAY eligibility is based on SECC 2011 data collected years ago. If he wasn't enumerated during the survey, or was missed due to migration, his name won't appear. He should approach the district health office for grievance redressal.

Q38. An NGO worker finds many eligible families in a village who have never heard of PM-JAY. What immediate steps should the NGO take?

➤ Conduct an eligibility check camp using the app, help families complete registration, coordinate with the nearest CSC or Ayushman Mitra, and arrange for immediate card generation for all identified eligible families.

★★★★ Level 4 – Tough (Problem Solving & Research)

Q39. Why is SECC 2011 considered an outdated basis for PM-JAY eligibility in 2024, and what reform would you propose?

➤ SECC 2011 is 13+ years old and misses millions born, migrated, or impoverished after 2011. A proposed reform: conduct a new SECC every 5 years, integrate with real-time databases (ration card, Jan Dhan, MNREGA), and create a dynamic beneficiary registry with annual updates.

Q40. A state government wants to add 20 lakh new families beyond SECC data. What legal, financial, and administrative steps are needed?

➤ Legally: issue a state government order defining new categories. Financially: the state must fund the expansion entirely since the Centre only funds SECC-based beneficiaries. Administratively: create a state-level beneficiary list, seed Aadhaar, and onboard them in the NHA portal.

Q41. From a research perspective, what are the inclusion and exclusion errors in PM-JAY's eligibility criteria?

➤ Inclusion errors (wrong people covered): urban middle class with forged documents, families who improved economically post-2011. Exclusion errors (deserving people missed): post-2011 poor, migrants not surveyed in SECC, informal workers in new gig economy. Exclusion errors are more harmful for equity and public health outcomes.

Q42. How does PM-JAY's eligibility model compare to other countries' universal health coverage approaches?

➤ India uses a targeted approach (identifying specific poor groups) unlike countries like the UK (NHS — everyone covered) or Thailand (universal coverage scheme). India's approach is cheaper to implement but risks exclusion errors. Countries like Indonesia (JKN) and Philippines (PhilHealth) use social insurance models. India's challenge is transitioning from targeted to universal coverage.

STAGE 2 – Registration & Card Generation

Level 1 – Simple (Basic Concepts)

Q1. What is an Ayushman Card?

➤ A digital/physical health identity card issued to PM-JAY beneficiaries, enabling cashless treatment at empanelled hospitals across India.

Q2. Where can a person register for an Ayushman Card?

➤ At Common Service Centres (CSC), empanelled government hospitals, district health offices, or online via the Ayushman Bharat app.

Q3. What is eKYC?

➤ Electronic Know Your Customer — the digital identity verification process using Aadhaar OTP or biometric fingerprint/face authentication done during registration.

Q4. Is there any fee to get an Ayushman Card?

➤ Registration is free. Printing a physical card at a CSC may cost a nominal ₹2–5.

Q5. Can the Ayushman Card be downloaded on a smartphone?

➤ Yes. After successful eKYC, the card can be downloaded from the Ayushman Bharat app or beneficiary.nha.gov.in.

Q6. What is the role of an Ayushman Mitra?

➤ Ayushman Mitras are trained assistants posted at hospitals and CSCs to help beneficiaries with eligibility checks, registration, and card generation.

Q7. What app is used for Ayushman Card registration?

➤ The official 'Ayushman Bharat' app published by the National Health Authority (NHA) on Google Play and App Store.
➤ To Download the App: https://play.google.com/store/apps/details?id=com.beneficiaryapp&hl=hi&pli=1

Q8. How many members of a family can be registered under one family card?

➤ All members of the eligible family can be registered. There is no cap on the number of family members.

Q9. What is the website for PM-JAY beneficiary registration?

➤ Link: beneficiary.nha.gov.in

Q10. What is the first step to register online?

➤ Download the Ayushman Bharat app, open it, enter your mobile number, and verify with the OTP received.

Q11. Does the mobile number need to be linked to Aadhaar for login?

➤ No. Any mobile number can be used for app login. But for OTP-based eKYC, Aadhaar must be linked to a mobile number.

Q12. What details can be used to search for eligibility in the app?

➤ Aadhaar number, ration card number, or name with location details.

Q13. What happens after successful eKYC?

➤ The Ayushman Card is generated digitally and can be immediately downloaded from the app.

Q14. Can a person register for multiple family members at once?

➤ Yes. After completing their own eKYC, the family head can add and register other family members listed under the same family ID.

Q15. What is a Family ID in PM-JAY?

➤ A unique identification number assigned to each eligible family in the PM-JAY system, linking all family members under one record.

★★ Level 2 – Medium (Concept + Process)

Q16. What are the three types of eKYC verification available?

➤ 1. OTP-based: OTP sent to Aadhaar-linked mobile. 2. Biometric: fingerprint or iris scan at CSC. 3. Face authentication: live photo matched with Aadhaar database via app.

Q17. Why is face authentication preferred over fingerprint for elderly or manual labourers?

➤ Elderly people and manual workers often have degraded fingerprints that fail biometric matching. Face authentication uses live photo recognition and works reliably regardless of fingerprint condition.

Q18. What should a person do if their mobile number is not linked to Aadhaar?

➤ Visit an Aadhaar enrolment centre to link the mobile number, or use biometric/face authentication at a CSC instead of OTP-based eKYC.

Q19. Step-by-step: What is the complete online registration process via the app?

➤ 1. Download app → 2. Login with mobile + OTP → 3. Search eligibility by Aadhaar/ration card → 4. If eligible, start eKYC → 5. Verify via OTP or face auth → 6. Fill and confirm details → 7. Card generated → 8. Download card.

Q20. What documents should a person carry when visiting a CSC for registration?

➤ Aadhaar card (original + copy), ration card, mobile phone, and any state-specific documents like income or caste certificate.

Q21. What is the difference between an individual Ayushman Card and a family card?

➤ A family card has a single Family ID covering all members. Individual cards are generated for each member linked to the same family ID. The ₹5 lakh limit is shared across the family, not per individual.

Q22. What should be done if the Ayushman Card doesn't generate even after successful eKYC?

➤ Wait and retry after some time (server issue), visit CSC if the problem persists, contact helpline 14555, or raise a grievance on pmjay.gov.in.

Q23. How is Aadhaar linked to the PM-JAY database?

➤ During registration, Aadhaar is seeded (linked) into the PM-JAY beneficiary system via eKYC. This creates a verified digital identity that connects the person's Aadhaar with their PM-JAY family ID.

Q24. What happens if two family members share the same Aadhaar due to error?

➤ This will cause a system conflict. The issue must be resolved at the Aadhaar level first (UIDAI correction), after which PM-JAY registration can proceed normally.

Q25. How does registration work for people with no fixed address or homeless beneficiaries?

➤ They can use an alternate address like a shelter home or NGO address for correspondence. Aadhaar-based biometric verification is sufficient for identity — a permanent address is not mandatory for registration.

Q26. Can a family update their details (new members, changed mobile) after registration?

➤ Yes. They can visit a CSC or district health office with relevant documents to update family details. Newborns and newly married members can be added.

Q27. What is the validity of the Ayushman Card?

➤ The Ayushman Card does not expire as long as the family remains in the beneficiary database. However, the ₹5 lakh benefit resets annually each year.

Q28. How does the hospital verify the Ayushman Card during admission?

➤ The hospital checks the card + Aadhaar of the patient through the PM-JAY portal, which does real-time biometric or OTP verification to confirm identity.

Q29. What is the role of a CSC operator in registration?

➤ The CSC operator helps the beneficiary search eligibility, operates the biometric device for fingerprint/iris scan, completes eKYC on their behalf, and prints the physical card.

Q30. What is 'seeding' in the context of PM-JAY?

➤ Seeding refers to linking a beneficiary's Aadhaar number to their PM-JAY family record in the national database, ensuring accurate identity verification and preventing duplicate registrations.

★★★ Level 3 – Scenario Based

Q31. An elderly woman with no smartphone and no Aadhaar-linked mobile wants to register. What is the best path?

➤ Visit the nearest CSC or empanelled hospital. Ayushman Mitra will check eligibility manually, use a biometric device for fingerprint or face verification, complete registration on her behalf, and print her physical card.

Q32. Suresh registered successfully but 3 family members are missing from his card. What should he do?

➤ Visit CSC or district health office with Aadhaar cards of missing members and ration card. Request manual addition of members. If denied, call 14555 or file grievance at pmjay.gov.in.

Q33. A beneficiary did eKYC via OTP but the card shows wrong name spelling. What are the steps to fix this?

➤ The name on the card is pulled from Aadhaar. So first correct the name in Aadhaar (via UIDAI), then re-do eKYC in PM-JAY. The corrected Aadhaar name will reflect on the Ayushman Card.

Q34. A young man is eligible but refuses to share Aadhaar for privacy reasons. Can he still register?

➤ Aadhaar is mandatory for eKYC under PM-JAY. Without it, registration is not possible through the standard process. He can be counselled that Aadhaar data is encrypted and not shared with hospitals — only identity verification is done.

Q35. A CSC in a remote village has no internet. How can registration happen there?

➤ NHA has introduced offline/low-connectivity modes in the app for basic registration. Alternatively, a mobile registration van with satellite internet, or the beneficiary must travel to the nearest town CSC.

Q36. A beneficiary lost their Ayushman Card. What should they do?

➤ The Ayushman Card can be re-downloaded anytime from the app or beneficiary.nha.gov.in using their mobile OTP. A new physical card can also be printed again at any CSC.

Q37. A person's face authentication keeps failing during registration. What could be the reasons and solutions?

➤ Reasons: poor lighting, camera quality, or database photo mismatch. Solutions: try in better lighting, clean camera lens, try OTP or fingerprint method instead, or visit CSC for assisted biometric registration.

Q38. Two brothers living separately want individual cards under the same family ID. Is this possible?

➤ Yes. They can both have individual Ayushman Cards linked to the same Family ID, with separate eKYC for each. However, the ₹5 lakh annual limit is shared between the entire family, not separate for each.

★★★★ Level 4 – Tough (Problem Solving & Research)

Q39. From a systems design perspective, what are the biggest technical bottlenecks in the registration process and how can they be fixed?

➤ Bottlenecks: 1. SECC data mismatches causing failed searches. Fix: real-time data reconciliation engine. 2. Biometric failures in elderly/labourers. Fix: prioritise face auth. 3. Poor rural internet. Fix: offline registration mode. 4. Server downtimes during peak camps. Fix: cloud auto-scaling. 5. Lack of CSCs in remote areas. Fix: mobile registration vans.

Q40. How does the PM-JAY registration system prevent duplicate card generation for the same person?

➤ Through Aadhaar-based deduplication — since each Aadhaar is unique and biometrically verified, the system detects and blocks duplicate registrations. Even name variations are resolved as the Aadhaar number is the unique key.

Q41. Critically evaluate the use of Aadhaar as the sole identity backbone for PM-JAY registration. What are the risks?

➤ Risks: 1. Exclusion of those without Aadhaar (elderly, homeless, tribal). 2. Biometric failures in specific populations. 3. Privacy concerns. 4. System dependency — Aadhaar outages affect registration. Mitigation: alternative verification paths (ration card + photo), regular biometric data updates, strong data privacy frameworks.

Q42. If you were designing a next-generation PM-JAY registration system, what features would you include to maximise coverage and minimise fraud?

➤ Features: 1. AI-based face authentication with liveness detection. 2. Real-time family data sync with civil registration system (births/deaths). 3. Offline-first mobile app with sync when connected. 4. Blockchain-based card ledger to prevent duplication. 5. Grievance auto-escalation within 48 hours. 6. Multi-lingual voice-assisted registration for illiterate users.

STAGE 3 – Awareness & Outreach

Level 1 – Simple (Basic Concepts)

Q1. What is the purpose of Awareness & Outreach in PM-JAY?

➤ To ensure every eligible family knows about the scheme, understands their entitlements, and knows how to use the Ayushman Card when hospitalisation is needed.

Q2. Who are Ayushman Mitras?

➤ Trained grassroots workers deployed at empanelled hospitals and CSCs to help beneficiaries with registration, card generation, hospital navigation, and grievance resolution.

Q3. Name three ways the government spreads awareness about PM-JAY.

➤ TV and radio advertisements, gram panchayat level camps, SMS alerts to beneficiaries, wall paintings in villages, ASHA/ANM outreach, and the Ayushman Bharat app.

Q4. In how many languages is PM-JAY information available?

➤ Multiple regional languages including Hindi, Tamil, Telugu, Kannada, Malayalam, Bengali, Marathi, Gujarati, Odia, Punjabi and more.

Q5. What is the PM-JAY helpline number?

➤ 14555 — toll-free, available in multiple languages for queries, registration help, hospital location, and grievance reporting.

Q6. What is the role of ASHA workers in PM-JAY outreach?

➤ ASHA (Accredited Social Health Activists) are frontline community health workers who spread awareness about PM-JAY in villages, identify unregistered families, and accompany patients to hospitals.

Q7. What is an outreach camp?

➤ A temporary setup organised at villages, panchayats, or public spaces where CSC operators and Ayushman Mitras visit to check eligibility and complete registration on-spot.

Q8. What is the main goal of PM-JAY outreach in urban slums?

➤ To reach urban poor families who may be eligible but unaware, helping them register and understand how to use the card at nearby empanelled hospitals.

Q9. Who organises state-level outreach campaigns for PM-JAY?

➤ State Health Agencies (SHA) in coordination with district health offices, CSCs, and NHA.

Q10. What visual tools are used for PM-JAY outreach in low-literacy areas?

➤ Pictorial pamphlets, wall paintings (murals), flip charts, audio-visual vans, and puppet shows/street plays in some states.

Q11. Can a person complain if a hospital refuses to admit them despite having an Ayushman Card?

➤ Yes. They can call 14555 immediately, report on pmjay.gov.in, or contact their district SHA office. Hospitals refusing eligible beneficiaries face penalties including delisting.

Q12. What is PM-JAY Mera Swasthya app?

➤ A mobile app component of Ayushman Bharat that helps beneficiaries access health information, nearby hospital details, and scheme-related services.

Q13. Are outreach activities only done at village level?

➤ No. Outreach is done at village, panchayat, ward, district, and state levels — including urban areas like slums, construction sites, and markets.

Q14. What does 'saturation' mean in PM-JAY outreach?

➤ Achieving near 100% registration of all eligible families in a given area — every eligible family has an Ayushman Card and is aware of how to use it.

Q15. What role do local elected representatives (sarpanch, councillors) play in PM-JAY awareness?

➤ They act as trusted community voices, help organise outreach camps, announce the scheme at gram sabhas, and resolve local level confusion or resistance to registration.

★★ Level 2 – Medium (Concept + Process)

Q16. What key information must be communicated to beneficiaries during outreach?

➤ 1. They are eligible and it is completely free. 2. What diseases and treatments are covered (health packages). 3. Names of nearby empanelled hospitals. 4. How to use the card — show Aadhaar + card at hospital. 5. Helpline 14555 for any help.

Q17. How are Ayushman Mitras trained?

➤ State Health Agencies train them on: PM-JAY portal operation, eKYC and card generation process, patient grievance handling, health package explanations, and hospital navigation for beneficiaries.

Q18. What is the significance of outreach in reducing the gap between eligible and registered families?

➤ Despite crores being eligible, many remain unregistered due to lack of awareness, language barriers, and distrust. Outreach directly closes this gap by reaching people at the community level and building trust through face-to-face interaction.

Q19. How does outreach differ between rural and urban settings?

➤ Rural: door-to-door visits, panchayat camps, coordination with ASHA workers. Urban: focus on slums, construction sites, street vendor areas, and markets. Urban outreach needs to address higher mobility and anonymity of beneficiaries.

Q20. What is the importance of IEC (Information, Education, Communication) materials in PM-JAY?

➤ IEC materials (pamphlets, posters, audio-visuals) translate complex government scheme information into simple, language-appropriate, and visually accessible content that beneficiaries at all literacy levels can understand.

Q21. How does PM-JAY use SMS-based communication for outreach?

➤ Registered beneficiaries receive SMS alerts about their eligibility status, registration confirmation, card download links, nearby hospital information, and reminders to use their benefits — in their regional language.

Q22. What are common myths about PM-JAY that outreach workers need to address?

➤ Myths: 'It's too complicated to apply', 'Hospitals will not actually treat us for free', 'The card will expire', 'Only government hospitals are covered', 'We need to pay something upfront'. All these are incorrect and outreach workers must correct them.

Q23. What is the Deen Dayal Upadhyay Antyodaya Yojana's connection to PM-JAY outreach?

➤ DAY-NULM and DAY-NRLM (rural/urban livelihood missions) share community networks with PM-JAY. SHG (Self Help Group) networks are used to spread PM-JAY awareness among women, who are key household decision-makers for healthcare.

Q24. How does inter-departmental coordination help PM-JAY outreach?

➤ Linking PM-JAY with PDS (ration shop), MNREGA, Jan Dhan, and Ujjwala Yojana databases ensures that families already engaged with other government schemes are automatically reached for PM-JAY awareness and registration.

Q25. What is a 'beneficiary verification camp' and how is it different from a registration camp?

➤ A registration camp focuses on new registrations and card generation. A beneficiary verification camp checks existing cardholders, updates family member details, verifies contact information, and resolves card-related issues for already registered families.

Q26. What is the PM-PM-JAY Jan Arogya Diwas?

➤ A national-level awareness day where camps, health check-ups, and registration drives are organised across the country to maximise PM-JAY awareness and enrolment — usually coinciding with the scheme's anniversary.

Q27. How do religious and cultural gatherings help PM-JAY outreach?

➤ Large gatherings like melas, fairs, temple festivals, and kumbh melas are used to set up PM-JAY awareness stalls and registration camps, reaching large numbers of eligible people in a short time.

Q28. What metrics does NHA use to track outreach effectiveness?

➤ Cards generated vs eligible families, scheme utilisation rate (cards used for treatment), awareness survey scores, grievance volume and resolution rate, and empanelled hospital reach.

Q29. How are media campaigns designed for PM-JAY to reach maximum beneficiaries?

➤ Multi-channel approach: national TV (Doordarshan), regional TV channels, All India Radio in local languages, newspaper ads, social media, SMS blasts, and community radio — all tailored in regional languages with simple visuals.

Q30. What is the PM-JAY grievance redressal mechanism?

➤ Beneficiaries can call 14555, visit the SHA or CSC, submit online at pmjay.gov.in, or write to the district health officer. NHA mandates resolution within defined timelines. Persistent issues escalate to state and national levels.

★★★ Level 3 – Scenario Based

Q31. A village in Rajasthan has 300 eligible PM-JAY families but only 60 registered. As an outreach researcher, what would you do?

➤ Step 1: Survey unregistered families to understand barriers (awareness, distrust, logistics). Step 2: Organise a gram sabha with sarpanch and demonstrate the app. Step 3: Set up a camp with CSC operator and Ayushman Mitra. Step 4: Conduct door-to-door registration. Step 5: Follow-up to ensure cards are downloaded and families know how to use them.

Q32. A beneficiary received an SMS about PM-JAY but thinks it's a scam. How should an outreach worker handle this?

➤ Visit in person with official ID. Show official app and government materials. Offer to call 14555 together for verification. Never ask for money or OTP. Bring a local trust figure (ASHA, sarpanch) to build credibility. Demonstrate on-spot eligibility check.

Q33. Women in a conservative community refuse to share Aadhaar details during outreach. What is the approach?

➤ Deploy female Ayushman Mitras. Conduct women-only sessions. Explain that Aadhaar data is encrypted and no personal data is shared with anyone. Partner with local women SHG leaders to build trust. Allow male family head to complete the process on behalf of family if culturally needed.

Q34. A construction site in Hyderabad has 500 migrant workers from Bihar. Many are eligible but unregistered. Design an outreach plan.

➤ Plan: 1. Coordinate with site contractor for access. 2. Deploy bilingual Ayushman Mitras (Hindi-Telugu). 3. Set up a registration camp on Sunday (workers' off-day). 4. Bring biometric devices and printed materials. 5. Conduct group demo of the app. 6. Register all workers on-site. 7. SMS them their card download link.

Q35. A district has high registration but very low utilisation of PM-JAY cards. What outreach gap does this indicate and how to fix it?

➤ Gap: families have cards but don't know which hospitals are empanelled, what is covered, or how to use the card at the hospital. Fix: conduct hospital-use awareness sessions, distribute list of empanelled hospitals, train ASHA workers on patient handholding during hospitalisation, and share success stories from the community.

Q36. Tribal communities in a forested area of Odisha are unreachable by regular outreach methods. What special approach is needed?

➤ Use mobile health vans with satellite internet. Partner with tribal community leaders and local NGOs. Deploy tribal language-speaking Ayushman Mitras. Coordinate with state tribal welfare department. Conduct outreach during local weekly haat (market) gatherings where tribal communities naturally congregate.

Q37. An outreach worker notices that most people who register never use the card for treatment. How should they investigate and address this?

➤ Investigate through beneficiary interviews: Is there an empanelled hospital nearby? Do they know what is covered? Have they tried and faced rejection? Analysis may reveal hospital-side barriers. Address by: hospital awareness camps, patient escort programs, and direct helpline support during hospitalisation.

Q38. A newly empanelled hospital in a rural area is receiving no PM-JAY patients. How can outreach resolve this?

➤ Update the empanelled hospital list on the app immediately. Conduct village-level announcement of the new empanelled hospital. Place banners at the hospital entrance. Train hospital's Ayushman Mitra to actively identify and assist PM-JAY patients. Coordinate with local ASHA workers to refer patients.

★★★★ Level 4 – Tough (Problem Solving & Research)

Q39. Research shows PM-JAY utilisation remains low even after 5 years. Identify the systemic outreach failures and propose a comprehensive reform.

➤ Failures: One-time outreach without follow-up, Ayushman Mitras undertrained, language barriers unaddressed, hospital-side resistance, no feedback loop from patients. Reform: mandatory annual re-outreach, integrate PM-JAY outreach into ASHA performance metrics, real-time utilisation dashboards for district officials, patient satisfaction surveys, and community health navigator programs.

Q40. How would you design a rigorous research study to evaluate the effectiveness of PM-JAY outreach programs?

➤ Design: Randomised Controlled Trial across districts with varying outreach intensities. Primary outcome: change in registration and utilisation rates. Secondary outcomes: awareness score (survey), hospitalization rates, out-of-pocket expenditure. Methods: baseline survey, intervention (structured outreach), endline survey, difference-in-difference analysis. Control for confounders: hospital density, literacy rate, internet penetration.

Q41. Critically analyse why health scheme awareness campaigns in India often fail to reach the most marginalised groups.

➤ Key failures: campaigns designed by urban officials unfamiliar with beneficiary context, language mismatch (Hindi-dominant content in non-Hindi regions), digital outreach excluding non-smartphone users, one-way communication without community feedback, distrust due to past failed schemes, and failure to engage local cultural intermediaries (religious leaders, women's groups). Reform requires community-co-designed outreach with built-in feedback mechanisms.

Q42. If given unlimited budget and authority, how would you redesign PM-JAY's outreach system to achieve 100% saturation within 2 years?

➤ Plan: 1. Deploy one Ayushman Mitra per 500 families nationwide. 2. Integrate PM-JAY into every government touchpoint (ration shop, MNREGA payment, Jan Dhan banking). 3. AI-powered SMS in regional language for every unregistered eligible family. 4. Mobile registration vans in every block. 5. School-based awareness (children inform parents). 6. Real-time dashboard tracking registration by village. 7. District Collector responsible for saturation targets. 8. Incentivise ASHA workers for registrations. 9. National media campaign with local celebrity ambassadors. 10. Annual Jan Arogya Saturation Mission.